



PATIENT HANDOUT

Eating during a flare and adding foods back

Ulcerative colitis: a practical guide for adults

Vance Medical Ltd provides general information and community support for people affected by gastrointestinal conditions. It is not medical advice and is not a substitute for the care of your own healthcare team. Nothing on this site should be used to diagnose or treat a health problem or disease. Always speak to your GP, pharmacist, dietitian or other qualified healthcare professional before making changes to your diet, medication or treatment, and with any questions about a medical condition. If you think you may have a medical emergency, call 999 or NHS 111 straight away.

Eating during a flare and adding foods back

Ulcerative colitis · A practical guide for adults

Food changes may help you feel more comfortable and keep eating. They do not replace treatment for bowel inflammation. There is no food list that suits everyone.[1]

FIRST STEPS

Start with what you can manage

Keep eating your usual foods if they are comfortable. Try smaller, regular meals when appetite is low. A short-term change in texture or portion size may help; you do not have to remove every food below.[3,4]

Foods you might reduce temporarily

For people able to eat and drink while under their healthcare team's care. A stoma, bowel narrowing, recent surgery, pregnancy or other dietary needs may require individual advice. This is not a diet for children.

Get help for a severe flare

A severe flare needs medical assessment, not a diet-only approach. Contact your IBD team promptly about a suspected flare. Seek urgent help for very bad or bloody diarrhoea, fever, a fast heartbeat or dehydration. Call 999 or go to A&E for severe tummy pain, non-stop bleeding, or a lot of blood or large clots.[2,5]

Food or drink	An alternative to try
Bran, seeded bread, raw salads, skins, whole nuts and popcorn	Try smaller portions or softer options: white bread, peeled potato and well-cooked peeled vegetables. Only reduce fibre when it helps your symptoms or your team advises it.[3,4]
Large portions of beans, lentils, onion, garlic or cabbage	These may increase gas. Adjust the foods that affect you. A meal based on egg, fish, chicken or tolerated plain tofu is an alternative.[3]
Greasy, deep-fried meals or very rich sauces	Try baked or poached food and smaller amounts of fat. Do not remove all fat: it supplies energy.[3,4]
Very hot chilli or hot sauces	Use milder seasoning when it helps. You do not need to give up every herb or spice.[4]
Caffeine, alcohol or fizzy drinks	Reduce them if they worsen diarrhoea or bloating. Try still water or a tolerated decaffeinated drink.[4,5]

Food or drink	An alternative to try
Sugar-free products with sorbitol, mannitol or xylitol	Check the ingredients. Reduce products that seem to cause loose stools or gas. Not every sweetener has the same effect.[4]
Milk or yoghurt, when lactose causes symptoms	Try lactose-free dairy or a suitable calcium-fortified alternative. Keep regular dairy if it suits you. Lactose-free milk is not safe for a milk allergy.[1,3]

DURING A FLARE

Keep nourishment in

More possible swaps, only where they help you.

Avoid unnecessary bans

Gluten, all dairy, tomatoes, onion and garlic do not need to be excluded just because you have colitis. Get advice before cutting out a whole food group. A low-FODMAP diet is not a treatment for a severe flare; it needs a trained dietitian when appropriate.[1,3]

Build a manageable meal

Try a protein food such as egg, fish, chicken or tolerated tofu; a starchy food such as white rice, pasta or peeled potato; and a small portion of soft fruit or vegetables if tolerated. Add snacks and extra portions as needed.[3,4]

Remember fluids

Sip regularly. Diarrhoea can mean you need extra fluid and salts. Ask a pharmacist or your IBD team about oral rehydration solution and follow its mixing instructions. Dark urine, peeing less or persistent dizziness need attention. Follow personal fluid advice, especially with a stoma, heart or kidney condition.[5]

Ask for support early

Contact your team or an IBD dietitian if you are losing weight, can eat only a few foods, or are struggling to keep enough food or fluid down. Do not replace meals with clear fluids or fasting on your own.[3,4]

AS THE FLARE SETTLES

Add foods back gradually

The aim is a varied diet, not a permanent avoidance list.

- 1 Start when you are improving.** Follow your treatment and any dietary advice from your team. There is no set number of days to stay on a gentle diet. A stoma, bowel narrowing or recent surgery may need a different plan.[6,7]
- 2 Try one change at a time.** Choose a small portion of one food you have missed, alongside a familiar meal. Note the portion, preparation and any symptoms. Increase gradually before adding several new foods together.[6]
- 3 Build up texture and variety.** Try more soft cooked fruit or vegetables first if these suit you. Gradually add oats, mixed white and wholegrain foods, then other plant foods you enjoy. No single order is compulsory.[6,7]
- 4 Keep protein, calcium and energy in view.** Continue regular meals and snacks. Ask for dietitian support if you have lost weight, feel afraid to eat, or can manage only a few foods. Ask your team about iron, vitamin D or other checks rather than starting high-dose supplements yourself.[3,7]

Examples, not a fixed timetable

Food	Starting point	A next step to try
Vegetables	Mashed carrot	Soft carrot pieces; then other cooked vegetables
Grains	White rice or pasta	Small portions of oats, or a little wholegrain mixed into white
Pulses	A pause if large portions caused gas	A small amount of smooth lentil soup or mashed beans
Milk	Lactose-free milk, when needed	A small amount of usual milk with food, if your team agrees

Try a smaller amount or a softer version if mild symptoms recur, and discuss repeated problems with your team. Do not retry a known food allergy. New bleeding, increasing pain or worsening diarrhoea needs medical advice, not just another food restriction.[2,6]

YOUR OWN PLAN

Make a note, not a ban

Use a short record to discuss patterns with your team.

One difficult meal does not prove an intolerance. Note what you ate, how it was prepared and any symptoms. There is no need to count every gram.[1,6]

Date / time	Food, amount and preparation	Symptoms and timing
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My next step

One food or texture to try: _____

IBD team / dietitian contact: _____

My agreed flare plan / notes: _____

GO FURTHER

More Support From VanceHealthHub

- **The IBD Flare Survival Guide:** practical steps for the first 48 hours of a flare
- **The Practical Food Guide for Crohn's & Colitis:** the wider principles, and when to see a dietitian
- **Health Record:** log symptoms, bowel pattern and dates in one private place
- **VANCE-Ai:** a starting point for questions between appointments, not a substitute for your care team

None of these replace your GP, gastroenterologist or dietitian, and nothing on VanceHealthHub should be used to diagnose or treat a condition.

REFERENCES

Sources and further reading

Do not wait for a food change to work

For a suspected flare, contact your IBD team. Seek urgent advice for very bad or bloody diarrhoea, fever, a fast heartbeat or dehydration. Call 999 or go to A&E for severe tummy pain, non-stop bleeding, or a lot of blood or large clots.[2,5]

Sources accessed 11 September 2026. General information, not a substitute for advice from your own healthcare team.
Version 1.0 · Prepared September 2026 · Continue medicines as prescribed and follow your own healthcare team's advice.

1. Crohn's & Colitis UK. Food.
2. NHS. Ulcerative colitis: when to get medical help.
3. University College London Hospitals NHS Foundation Trust. Ulcerative Colitis: My Diet During a Flare Up.
4. Crohn's & Colitis Foundation. What Should I Eat with Crohn's or Colitis?
5. Crohn's & Colitis UK. Dehydration.
6. Calderdale and Huddersfield NHS Foundation Trust. Reintroducing Fibre into your Diet.
7. University College London Hospitals NHS Foundation Trust. Ulcerative Colitis: My Diet during remission.

Medical disclaimer: general information only, not a substitute for advice from your own healthcare team.
For a life-threatening emergency, call 999 or go to A&E. If you need urgent medical advice and it isn't an emergency, contact NHS 111.